FINAL DRAFT

Design Note: New image to be inserted for the cover of the plan

Qatar National Mental Health Plan 2025-2030

October 2025

وزارة الصحـــــة العامـــــة
Ministry of Public Health
State of Qatar • دولـــة قطـــر

Acknowledgements

We extend our thanks to all of those who supported the development of this plan. Representatives from organizations across the mental health system have provided their expertise and knowledge, guiding the creation of this document.

Special thanks go to members of the National Mental Health and Wellbeing Committee and to the National Mental Health Program for their support in coordinating and preparing the final plan.

TABLE OF CONTENTS

		Page
Forewords		3
His Excellency Mansoor bin Ebrahim bin Saad Al Mahmoud.		
Sheikh Dr Mohamed Al Thani Director of Non-Communicable Diseases Prevention Programs		4
Executive Summary		5
Section 1:	**Setting the Strategic Context:**	
	National Strategies and Plans	7
Section 2:	**Why we need a National Mental Health Plan for Qatar:**	
	Where we are now	11
	Our key challenges	19
	Our Ideal System & Future State	*25*
Section 3:	**Our Priorities for Mental Health**	
	Action Plan - Projects and Milestones	28
Section 4:	**How we will translate strategy into action:**	
	Measuring successful implementation	39
Annexes:	**Annex 1:** Acknowledgments	40
	Annex 2: Abbreviations	41
	Annex 3: References	42

FOREWORD

Foreword by His Excellency, Mansoor bin Ebrahim bin Saad Al Mahmoud

Minister of Public Health
State of Qatar

DRAFT TO BE APPROVED

It is my privilege to introduce Qatar's third National Mental Health Plan 2025-2030.

Since the launch of the first Qatar National Mental Health Strategy (QNMHS) in 2013–2018, there has been significant progress in improving and developing the mental health system in Qatar.

This development can be largely attributed to the ambitious and visionary objectives outlined in several key strategic plans including the Qatar National Vision 2030 (QNV), the Qatar National Development Strategy (QNDS) and the past and current Qatar National Health Strategies (QNHS).

This new Plan for 2025-2030 will help our key stakeholders to embed the strategic Vision for mental health and wellbeing, which is: ***To create a society that values, prioritizes, and supports mental health and well-being, where individuals have access to comprehensive, integrated quality mental health services, stigma is reduced, and a culture of understanding and compassion prevails.***

A comprehensive action plan has been developed, working across multisector organizations with the joint aim of improving the mental health and well-being of all our communities.

I am fully committed to supporting the implementation of this Plan and continuing the successful collaborations across all sectors to achieve its objectives. By working together, we can create meaningful and a lasting impact, to build resilient and thriving communities across Qatar.

FOREWORD

Sheikh Dr Mohamed Al Thani - Director of Non-Communicable Diseases Prevention Programs

The past ten years have seen the mental health system transformed at all levels. The development of new integrated services within primary care and across the community has ensured easier access to mental health support in non-stigmatized environments, making it easier for people to seek help early.

The establishment of a dedicated National Mental Health Helpline has enhanced access to services, completed by the introduction of specialist virtual psychological and psychiatry services. The focus on promoting a preventative mental health agenda has also seen a significant shift in attitudes and awareness towards mental health.

As a truly integrated health system we want to deliver the best possible care for those affected by mental illness. This means ensuring individuals have access to high quality, integrated mental health services both in the community and across our specialist inpatient mental health services.

We will continue to raise awareness about mental health and reduce stigma associated with mental illness. We will continue to strive together to enhance everyone's mental health and wellbeing through an integrated model of care. In doing so we will deliver the best outcomes possible for the whole population of Qatar.

The Plan was developed through stakeholder consultation and focuses on achieving the priority outcomes set out in the third National Health Strategy 2024-2030. It also identifies key initiatives and projects, aimed at advancing mental health and substance use disorder treatment services. This is to ensure we deliver comprehensive integrated care that meets the needs of individuals, their families and carers.

EXECUTIVE SUMMARY

The National Plan for Mental Health, aligned with the key priorities and outcomes identified in Qatar National Health Strategy 2024-2030 (QNHS 2024-2030), focuses on three significant areas: **improving access**, **delivering high-quality services**, and **enhancing the efficiency and resilience** of Qatar's mental health treatment system. Additionally, specific milestones for developing the substance use disorder (SUD) treatment system have been incorporated into this Plan.

Improving Access

The plan outlines practical steps to enhance access to mental health treatment from various entry points. Key initiatives include implementing a 'digital gateway' to provide comprehensive mental health prevention and wellbeing information and education, improving access to the mental health helpline through digital means and hospital emergency departments, and expanding mental health services within primary care health centers.

The latter includes further integrating mental health specialists within health centers to reduce stigma and improve accessibility.

Delivering High-Quality Mental Health Services

The plan emphasizes the consolidation and enhancement of specialist mental health services and integrated care pathways between PHCC and secondary care mental health services provided by HMC. Key developments include establishing new services as outlined in the national integrated mental health facilities and bed plan.

These include child and adolescent inpatient services, psychiatric decision units within general hospitals, dedicated inpatient services for people with intellectual disabilities, a forensic mental health hospital, long-term residential care for patients with severe enduring mental illnesses, an acute mental health unit for older adults, a perinatal mental health unit, and a crisis/home treatment service.

Additionally, the plan aims to improve the provision of evidence-based psychological therapy services through a National Program on access to psychological services.

Enhancing Efficiency and Resilience

The plan aligns with health system efficiency outcomes and initiatives identified in QNHS 2024-2030, focusing on mental health specific program-based budgeting, financial optimization, and data-driven intelligence to navigate patient care pathways and ensure appropriate care based on clinical need. Mental health research will drive culturally adapted innovative interventions contributing to all three key priorities.

Upskilling the workforce in delivering high-quality mental health services through support mechanisms, career pathways, and flexible working models will further enhance efficiency and resilience within the mental health treatment system, including staff retention.

Furthermore, projects have been established for the continued development of the substance use disorder treatment system, encompassing the creation of national plans for the prevention and treatment of SUDs.

Measuring implementation of the Plan

To effectively deliver the outcomes of the National Mental Health Plan, a detailed implementation plan will be developed with key deliverables and performance indicators. Progress will be regularly monitored by the relevant oversight structures within the QNHS 2024-30.

In order to show the direction of the transformation we want to achieve; this Plan is divided into four key sections:

Section 1: Outlines the strategic and national context and show the alignment to QNHS 2024-30.

Section 2: Highlights why we need a National Mental Health Strategic Plan for Qatar, where we are now and where we need to be by 2030.

Section 3: Provides details of our key priorities and project deliverables aligned to QNHS 2024-30 over the next five years. These will be delivered in collaboration with our key partners to ensure people have access to quality mental health services and support.

Section 4: Outlines how we will translate our strategic plan into action.

SECTION 1: SETTING THE CONTEXT

Strategic Context

The QNMH Plan 2024-2030 has been developed to ensure alignment to the Qatar National Vision 2030 (QNV), the Qatar National Development Strategy (QNDS) for 2024–2030, the Qatar National Health Strategy (QNHS 2024–2030) and the WHO Global Action Plan for Mental Health 2013-2030. Figure 1 below shows the alignment of these strategies related to mental health.

Design Note – figure to be designed

Figure 1: Strategic Alignment

Focusing specifically on the priority and outcome areas the Qatar National Health Strategy 2024-2030 (QNHS 2024-2030) outlined in ***Figure 2 below***.

Design Note – figure to be designed ***Figure 2: QNHS 2024-2030 Priority and Outcome Areas 2024-2030***

Our National Mental Health Strategic Plan 2025-2030 focuses on three main priorities:

- **Improving access**
- **Excellence in mental health service delivery**
- **Mental health system efficiency and resilience**

These priorities aim to address the growing mental health needs of our population through a comprehensive and integrated approach. The QNHS 2024-2030 specific projects related to mental health and substance use are addressed throughout this Plan and for each of the identified project there will be a dedicated project plan highlighting the key actions, timeframes, lead stakeholders as well as outcome measure and key performance indicators.

In addition, our Action Plan highlights the interdependencies related to mental health and substance use across the whole of QNHS 2024-2030.

Improving Access

This priority emphasizes the need to enhance the accessibility of mental health prevention, promotion, and services for the population, aiming to make access easier, more modern, and less stigmatizing. Key projects initiatives include:

- **Mental Health Prevention and Promotion**: We will implement a 'digital gateway' to provide comprehensive mental health prevention, wellbeing information, and education. This initiative will enhance our population's understanding and management of mental health through improved health literacy. Additionally, we will enable 24/7 access to the mental health helpline through digital means, making it easier for people to access services and reduce stigma. To support the preventative agenda, we will expand the National Mental Health Awareness Program Ma'ak.
- **Integrated Mental Health and Substance Use Disorder Treatment Services in PHCC:** Further expand primary care mental health screening and management of mental health conditions under the family medicine model and increase the number of integrated psychiatric clinics within health centers. Additionally, we will roll out the Screening, Brief Interventions, and Referral to Treatment (SBIRT) program for substance use disorders and co-locate substance use disorder specialists in health centers.
- **Emergency Departments:** Development of mental health-friendly environments within hospital emergency departments, and the provision of access to dedicated Psychiatric Decision Units within general hospitals.

- **Cross-Sectoral Collaboration**: We will engage with partners across various sectors to promote wellbeing and mental health. This includes collaborating with educational institutions, workplaces, community organizations, and governmental agencies to develop and implement comprehensive mental health initiatives. By working together, we aim to create a supportive environment that enhances mental health awareness, reduces stigma, and provides integrated support systems.

Excellence in Service Delivery

Our strategic plan emphasizes the enhancement of mental health services through the development of primary and specialized mental health care, integrated service delivery, and patient-centered approaches. This plan aims to ensure high-quality care across all population groups, promoting overall mental wellbeing and addressing specific needs through targeted initiatives. Our plan involves further development of primary health care and specialist mental health services, focusing on delivering high-quality mental health services across all levels of care. Key elements include:

- **Consolidation of Specialist Mental Health Services and Development of Integrated Pathways:** We will consolidate geographically dispersed specialist mental health facilities within HMC and integrate care models across primary, secondary, and long-term residential care settings to ensure seamless patient journeys.
- **Integrated Child & Adolescent Mental Health Services:** We will further expand Child and Adolescent Mental Health services in primary and secondary care settings and ensure easier access and integration.
- **Patient-Centered Care:** Ensuring that care is tailored to individual needs, preferences, and values, with an emphasis on patient empowerment and engagement.
- **Primary and Community Care Excellence:** Strengthening primary and community care as the foundation of the mental health system, ensuring services are effective, safe, timely, efficient, and equitable.
- **Development of Sub-Specialties:** We will focus on developing much-needed dedicated sub-specialty inpatient and outpatient services, including child and adolescent mental health services, intellectual disability mental health services, perinatal mental health services, healthy ageing mental health acute inpatient services, and forensic mental health services.
- **Access to Psychological Therapies:** We will develop our psychology workforce and roll out a national program to improve the accessibility and delivery of evidence-based psychological therapies, making these therapies available both in person and through virtual and digital means.

Mental Health System Efficiency and Resilience

This priority addresses the need for a dynamic and responsive mental health system capable of meeting current and future challenges. Key initiatives include:

- **Workforce Planning and Development:** Our focus will be on the recruitment and retention of staff, continuous education, professional development, and flexible working models to enhance workforce wellbeing and satisfaction.
- **Data-Driven Decision Making:** We will implement robust data governance and analytics capabilities to support informed decision-making. This includes a unified mental health data set across primary and secondary care, with performance and outcome monitoring.
- **Sustainable Healthcare Financing**: Given that mental health services will not be part of the upcoming National Health Insurance, we will shift towards program-based, ring-fenced budgets to ensure the financial sustainability of mental health services, aligning sector financing with strategic objectives.
- **Innovation and Research:** Fostering a culture of research and development to drive innovation in mental health care, including expanding mental health research and genome sequencing capabilities.
- **Right Care in the Right Location:** Achieving efficiencies by ensuring the majority of mental health care is community-based. Stable patients with severe mental illness will be treated in primary care health centers and specialist community mental health hubs, supported by appropriate resources.

By focusing on these priorities, the Qatar National Mental Health Strategic Plan aims to contribute to a healthy society supported by an integrated health system that prioritizes clinical excellence, sustainability, and innovation. This approach ensures that Qatar is well-positioned to address the mental health needs of its population both now and in the future.

SECTION 2: WHERE WE ARE NOW & WHERE WE WANT TO BE BY 2030

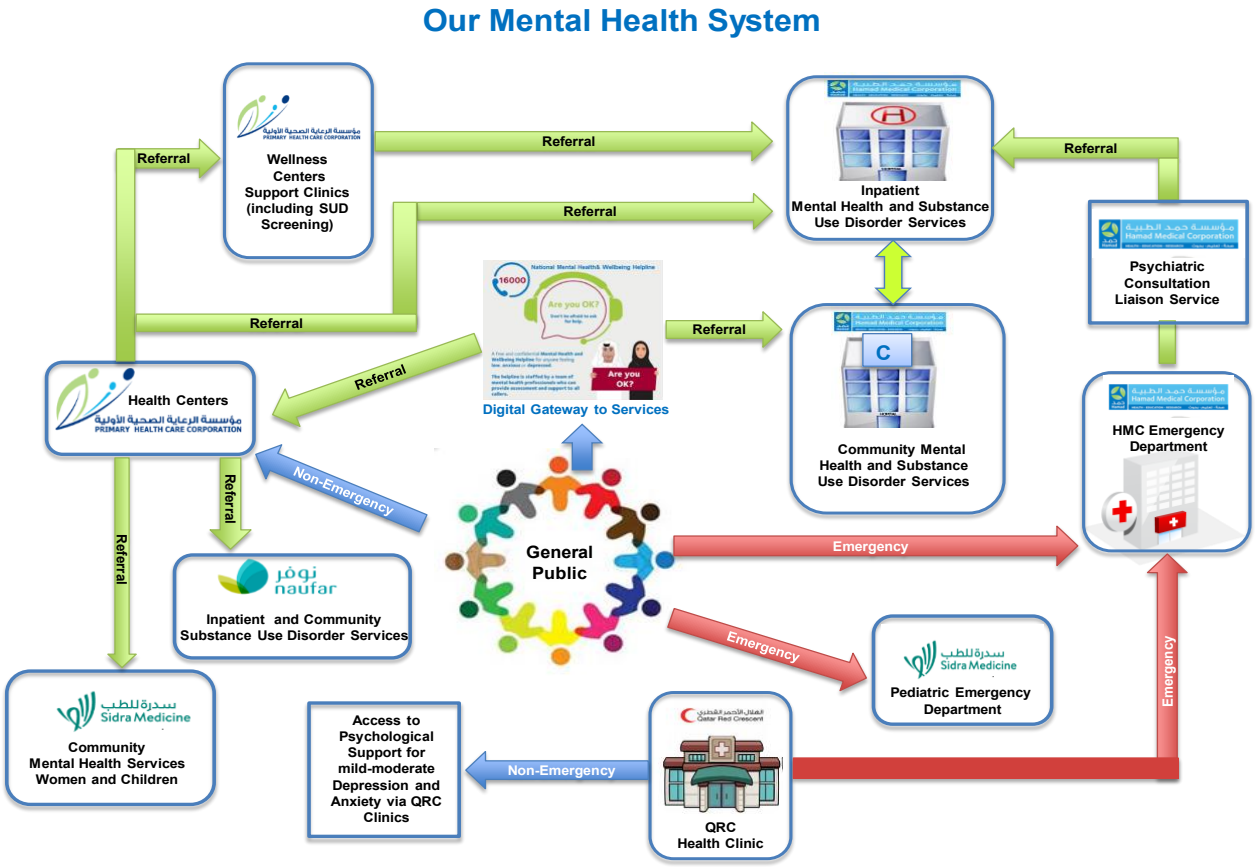
Where are we now

The Qatar National Health Strategy 2018-22 (NHS2) and the National Mental Health and Wellbeing Framework 2019-22 established clear priorities for developing community and primary care-based mental health services, underpinned by specialist inpatient beds.

Over the last several years, significant progress has been made in expanding primary health care mental health services, specialist community mental health services, and integrated mental health services across Hamad Medical Corporation's hospitals and Primary Health Care Corporation's health centers. This includes establishing a dedicated National Mental Health Helpline and introducing specialist integrated psychiatric clinics within primary care health centers.

Depicted below in figure 3 is our current mental health system:

Design Note – figure to be designed ***Figure 3: Our current mental health system***



PHCC has made significant strides in integrating the management of mental disorders and substance use disorder screening, brief interventions, and referral to treatment (SBIRT) services into the family medicine model.

Following the training of family physicians and nurses in managing mental illnesses, there has been a notable increase in the number of patients treated for mental disorders.

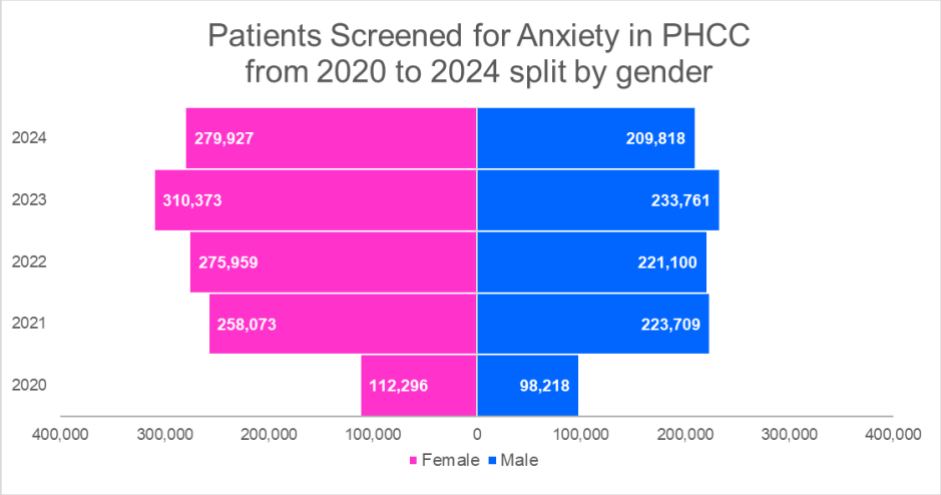
- Over 580 family physicians have been trained to manage common mental illnesses and in SBIRT.
- Over 1600 nurses trained to screen for depression and anxiety.

Charts 1-4 below outline the significant trends and expansion of primary care mental health services over the last several years.

Design Note – all charts to be designed

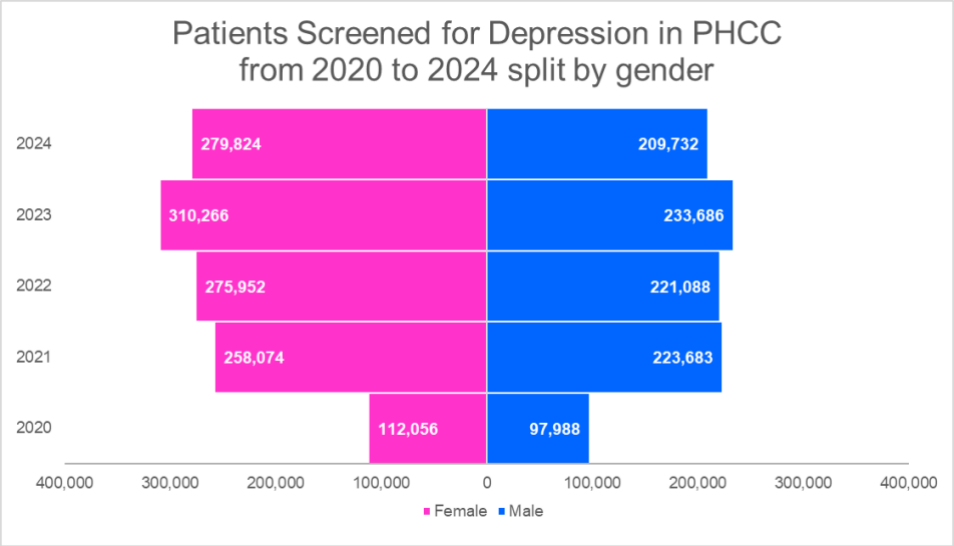
Chart 1 below shows the number of adults screened for Anxiety in PHCC between 2020 and 2024 split by gender – the figure shows an increase in screening year-on-year particularly for females.

Chart 1: Patients screened for anxiety in PHCC from 2020 to 2024 split by gender



The depression screening figure below shows similar results to the anxiety screening and follows the same trend for females.

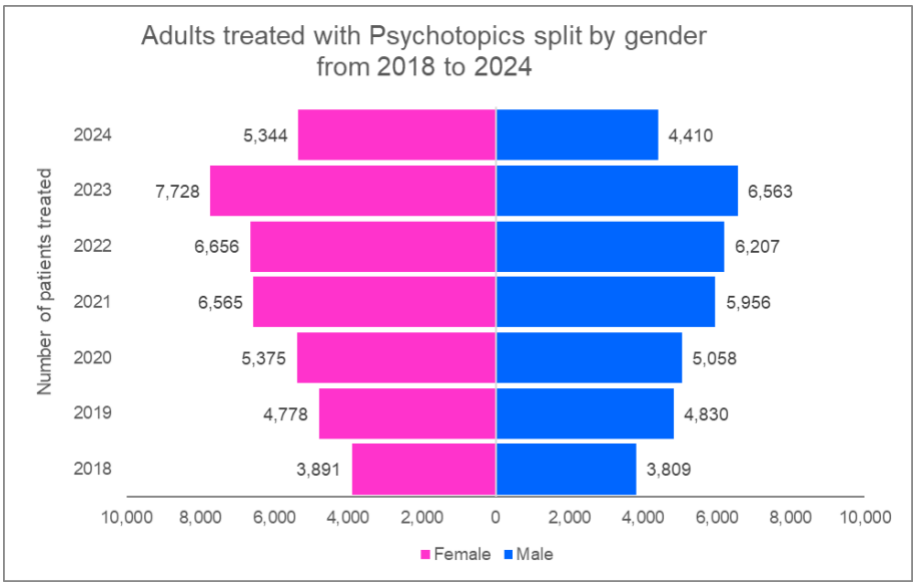
Chart 2: Patients screened for depression in PHCC from 2020 to 2024 split by gender



The screening data above indicates that screening efforts have been integrated into routine practice within health centers, with over half a million adults being screened for common mental illnesses in 2023.

Outlined in Chart 3 below the figures show the number of adults treated with psychotropic medication for mental health condition from 2018 to 2024 split by gender.

Chart 3: Adults treated with psychotropic medication for a mental health diagnosis

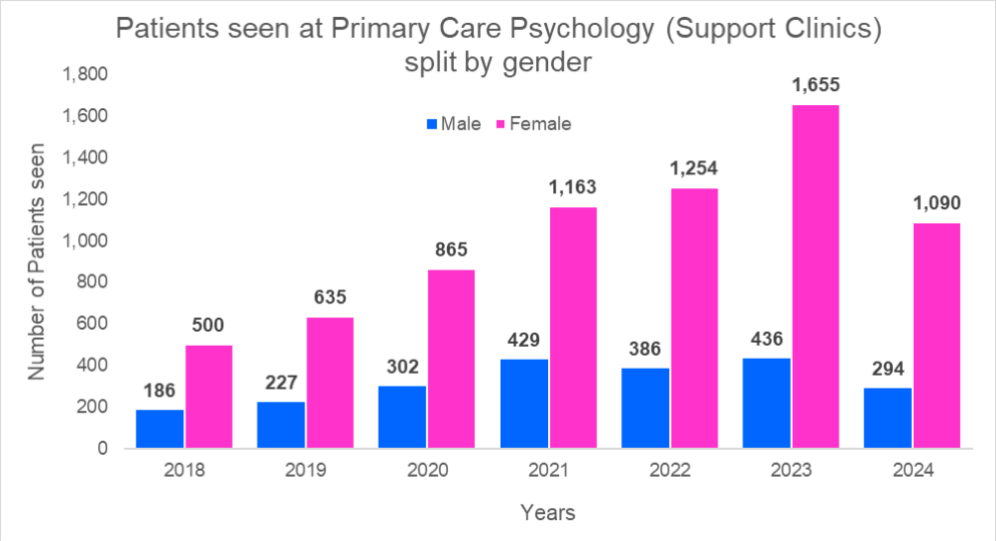


Over a 6-year period, the number of adults treated for mental disorders requiring psychotropic medication significantly increased, reaching 14,291 adult patients in 2023. The most common diagnoses treated and managed include mood disorders, depression, anxiety, panic disorders, phobias, eating disorders, obsessive-compulsive disorders, stress disorders, sleep disorders, ADHD and behavioral disorders, schizophrenia, and dementia.

In Chart 4 the number of patients seen in primary care psychology services, commonly known as Support Clinics, has steadily increased. While the attendance rate for Support Clinics was 78% in 2022, it was 68% in 2023. Support clinics are currently available in 9 health centers, ensuring patient access from all regions in Qatar.

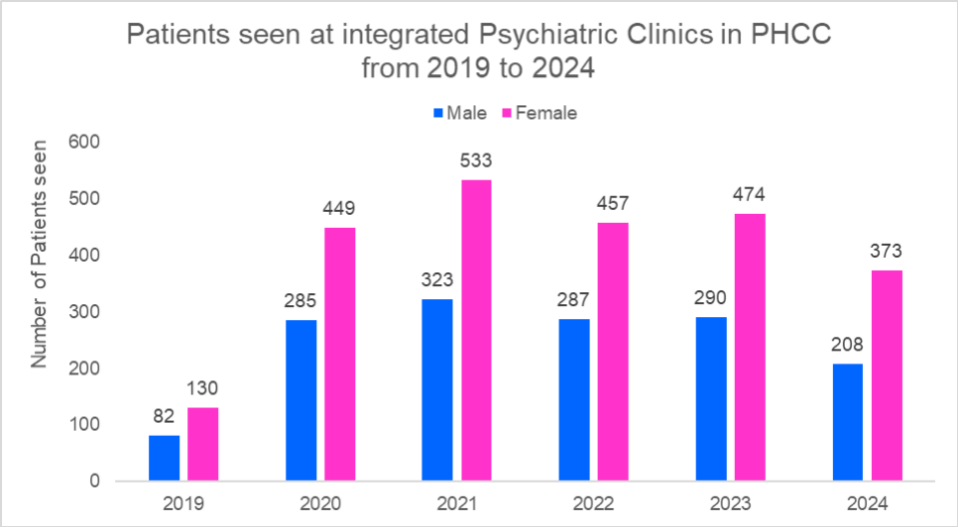
This is in line with the increase in screening rates and hence referrals to the Support Clinics. The number however shows a drop in 2024, and this is highly related to data completeness issues.

Chart 4: Patients seen at Primary Care Psychology (Support Clinics) split by gender



PHCC's Integrated Adult Psychiatry service is designed to support and manage patients with moderate to severe mental health disorders. Chart 5 shows the number of patients seen in the 7 Adult Integrated Psychiatry Clinics at PHCC between 2019 and 2024.

Chart 5: Patients seen in integrated psychiatric clinics at PHCC



Charts 6-9 below highlight the growth seen across secondary care specialist mental health services provided by HMC during and immediately after the COVID period.

Over the last 5 years, due to increased demand, there was a significant rise in the number of patients seen in outpatient clinics, referrals to psychiatric consultation liaison teams in general hospitals, inpatient discharges, and the number of patients managed in the community, including those who received home visits.

The figure in Chart 6 shows the number of outpatient clinic data between 2019 and 2024. The total number visits in the Outpatient department and Mental Health Services shows an increase every year, and similarly the number of new visits to mental health services shows an increase since 2019. The percentage of new visits is between 12%-13% of total visits to OPD mental health services.

Chart 6: HMC secondary care new visits and total visits in OPD-MHS

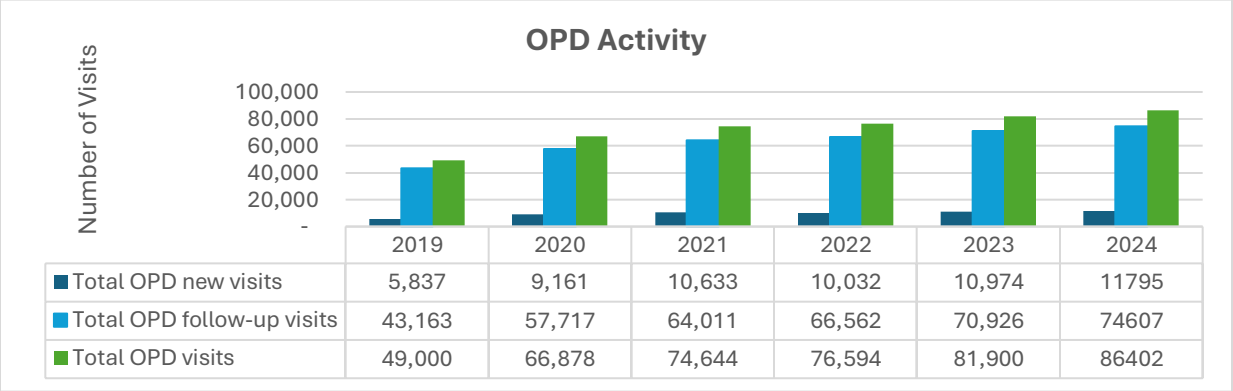
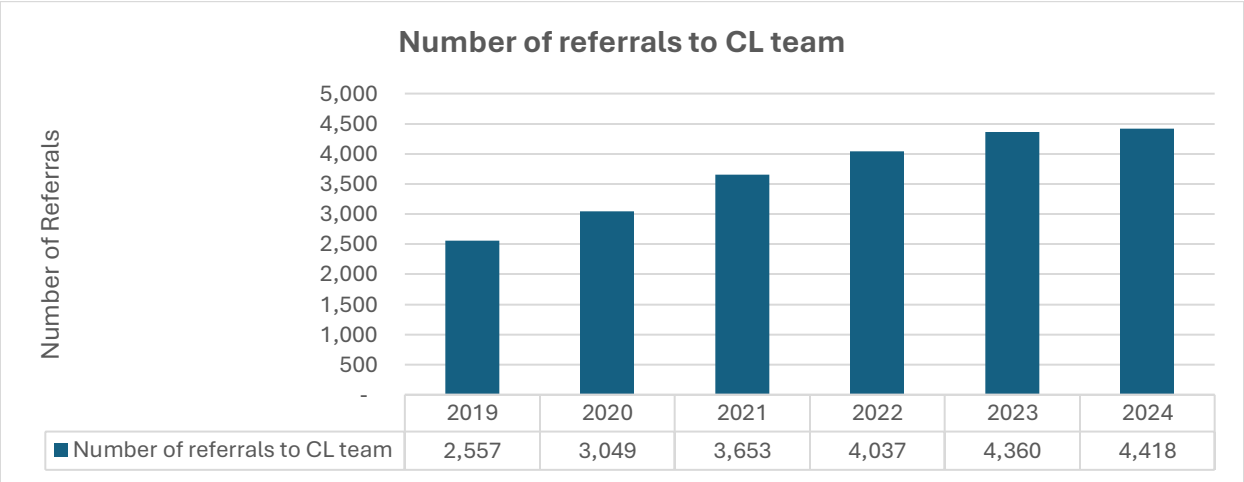


Chart 7 figure below shows the referrals to Psychiatric consultation liaison (CL) team at HMC emergency department. There is a constant increase in the referrals to CL services around 6% increase between 2023 and 2024.

Chart 7: Number of Referrals to Psychiatric CL Team in ED-HMC



In relation to Chart 8, this demonstrates over the past 6 years more patients are being discharged into community services. Chart 9 outlined the increase in the number of patients receiving those services either via home visits or other mental health community facilities.

Chart 8: Inpatient discharges 2019-2024

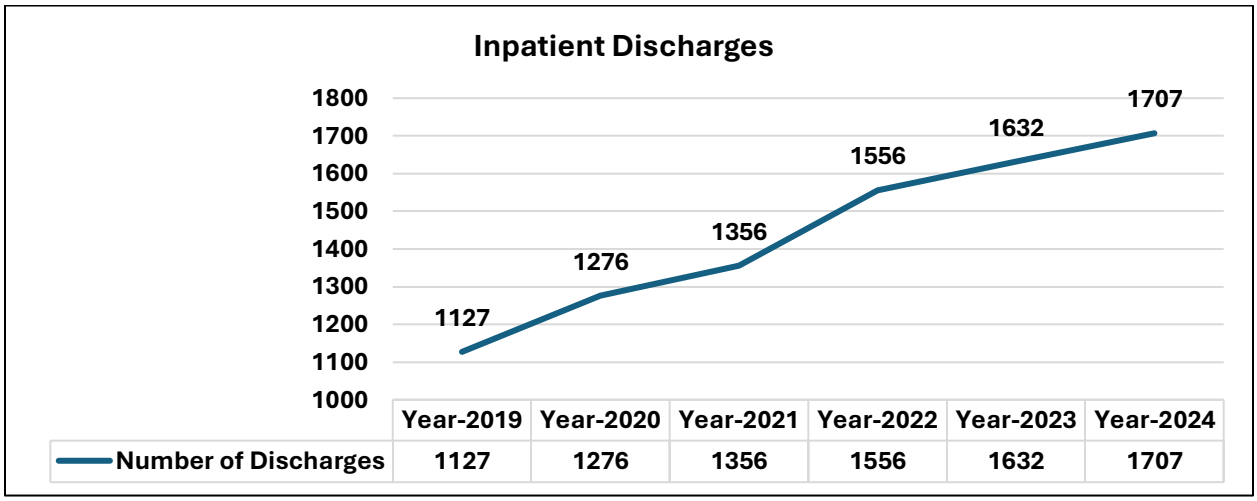
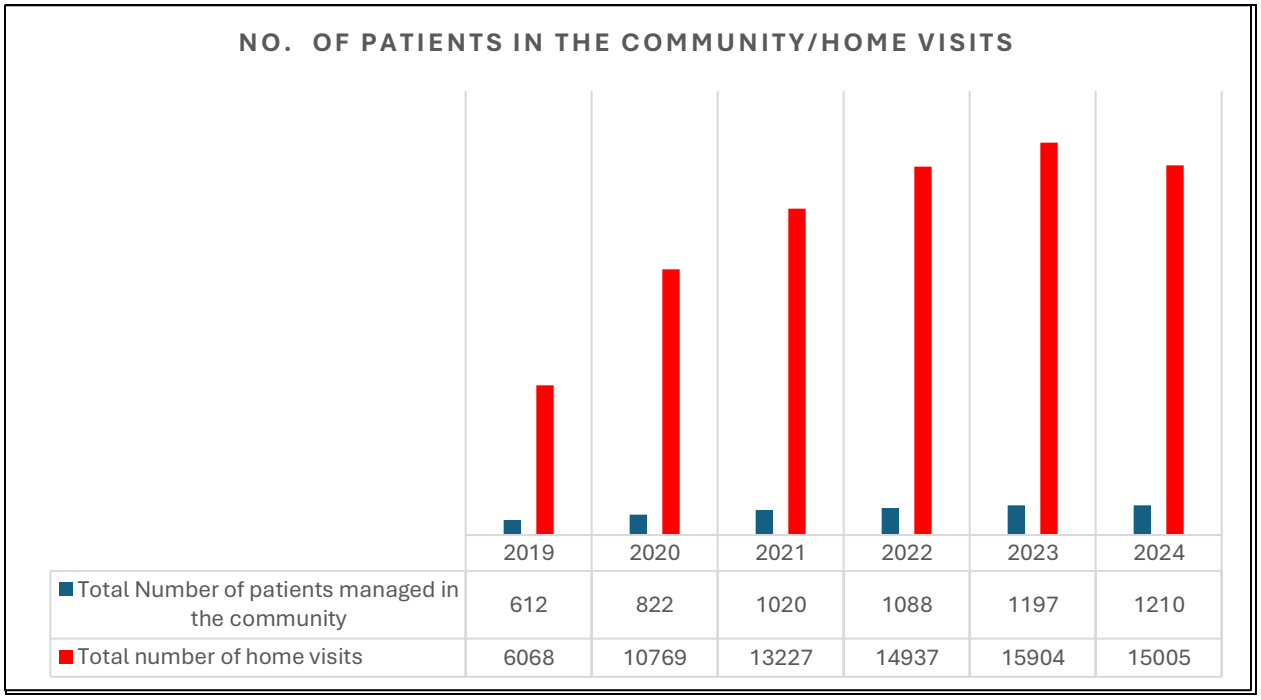


Chart 9: Patients managed in community/ home visits from 2019 to 2024

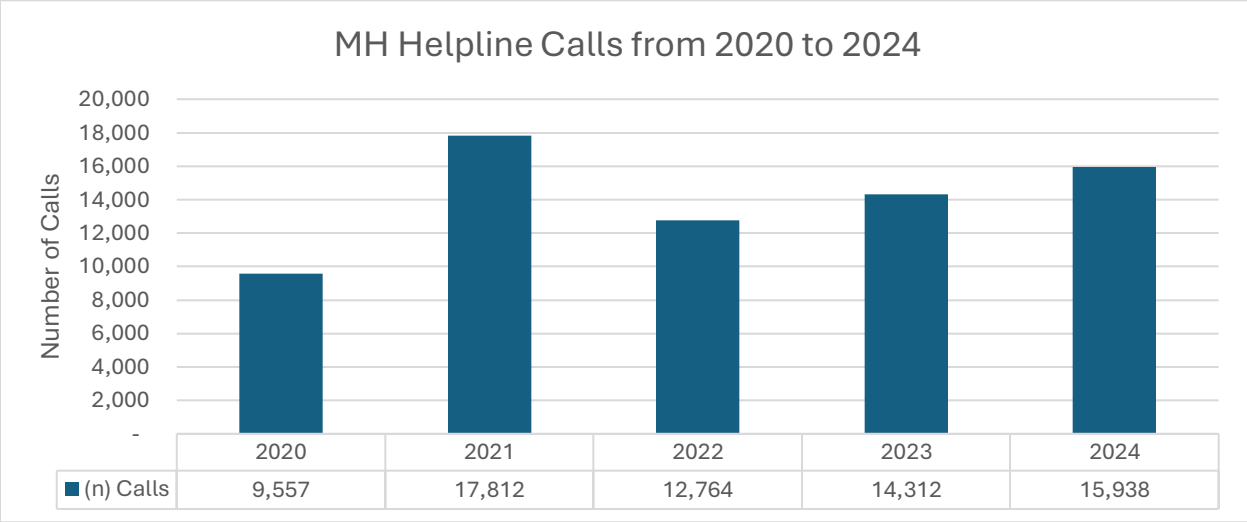


Furthermore, since the launch of the National Mental Health Helpline during the COVID period, over 70,000 calls have been managed until the end of 2024.

The helpline continues to operate, providing the public with confidential advice, support, treatment, and referral to other services.

It also provides specialist mental health support for callers, including emergency calls, who are assessed as high risk. In 2022 **426** high risk calls received out of which **223** (52%) presented to Emergency Departments. The remaining 48% were not referred to ED and were managed and supported by the NMHH, effectively reducing potential attendance at ED.

Chart 10: Helpline Calls



An area of best practice has been the development of virtual women's services, including specialized perinatal mental health support. The National Mental Health Helpline provides a dedicated virtual women's service, providing support to female callers and a single point of access for perinatal referrals across the system.

Since the development of the service there has been in increase in the level of need for specialist perinatal mental health support with calls to the National Mental Health Helpline steadily increasing till 2023 as seen below:

Chart 11: Perinatal Cases Managed via the Helpline

Year	Perinatal Total Cases (Helpline)
2021	95
2022	143
2023	243
2024	161

Referrals from professionals for perinatal support, to the single point of access through Cerner have also increased, with a total of 684 referrals in 2023 and 674 referrals recorded from January to August 2024. Activity in both services highlight the increasing number of women and their families receiving specialist perinatal mental health support.

In March 2021, HMC's mental health services developed The Women's Wellness Centre (WWC) a 12 bedded in patient facility for female patients who are recovering from an acute episode of mental illness.

Currently perinatal women in acute need can access these services if they are in low to medium need of inpatient support. The service also provides joint support alongside the Women's Wellness and Research Center (WWRC) for women in the maternity wards with specialist one-one-one care being provided by mental health service professionals.

Our Challenges

Despite our progress, we face several challenges, particularly in meeting the post-COVID demand for mental health services across community, primary, secondary, and specialist mental health services. Additionally, there are system-wide challenges that need to be addressed.

Community Challenges

- **Stigma**: Public campaigns during and after COVID have significantly increased awareness of mental health and reduced the stigma associated with mental illness. This impact is witnessed in the national mental health attitudes and awareness survey index, which shows a significant positive shift across all four areas of the index.

Design note – Figure to be designed

QMHAAI Total Weighted Index Measure			
	2018	2020	2022
MH Communications	4.31	6.27	6.32
MH Awareness	20.57	23.87	24.06
MH Attitudes	19.09	21.04	21.64
MH Services	11.50	12.80	14.82
Total Weighted Index	55.47	63.98	66.84

However, attitudes towards mental illness are still a barrier that prevents many individuals from seeking help. Ongoing efforts across specific population groups are needed to tackle stigma, and we will continue to develop effective campaigns to address this issue.

- **Peer Support:** The availability of culturally appropriate peer support groups and networks for individuals with mental health conditions is limited. These groups are necessary for providing emotional support, fostering a sense of community, and sharing experiences among individuals facing similar challenges. They can significantly enhance mental health outcomes and promote recovery.

- **Geographical Barriers:** HMC's specialist inpatient and outpatient mental health services are currently dispersed geographically. The majority of inpatient services are located in Mesaieed Hospital, in the South of Qatar, while sub-specialties and outpatient services are spread across Doha and other regions. This distribution poses challenges for convenient public access to these services.

Primary Health Care Challenges

- **Post-COVID Surge in Demand**: There has been a significant increase in demand for mental health services post-COVID, with a notable rise in depression and anxiety, as evidenced by the increasing number of patients treated in PHCC for common mental illnesses.
- **Expansion of Support Clinics:** There is a need to expand primary care psychology services to ensure that a large majority of patients prescribed psychotropic medications also receive psychological interventions, in line with the stepped care model. The waiting times for psychology services at PHCC are currently over six weeks.
- **Expansion of Child & Adolescent Mental Health Clinics**: There is a need to expand integrated child and adolescent mental health clinics in PHCC to ensure enhanced access to such services closer to home. This includes expanding the current child and adolescent clinic (Student Advisory Clinic) to have one clinic in each region. Additionally, there is a need to ensure the integration of care and embedding integrated pathways across the system.
- **Waiting Times for Psychology and Integrated Psychiatry appointments:** PHCC data from 2023 indicates that 42% of non-Qatari patients and 14% of Qatari patients waited over 8 weeks for an appointment at psychology support clinics. Additionally, data reveals that 86% of non-Qatari and 59% of Qatari patients waited over 8 weeks for an appointment at integrated psychiatric clinics. These extended wait times hinder timely access to appropriate care, placing additional strain on other parts of the healthcare system, including secondary care and emergency services.

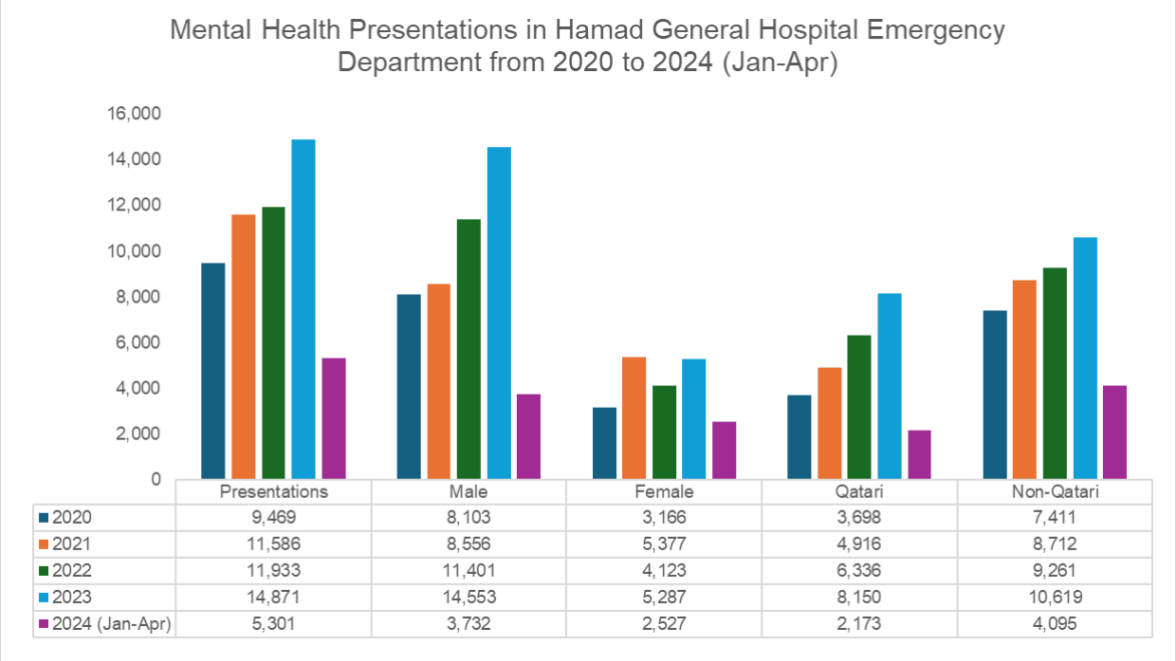
- **Expansion of Integrated Psychiatric Clinics:** There is a need to expand integrated psychiatric clinics in PHCC to ensure easier access to care closer to patients' homes. This includes shifting more specialist outpatient clinics from HMC to primary care health centers.
  Additionally, further integration of care models is necessary to better distribute resources, reduce waiting times, and reduce the stigma associated with mental illness. This approach will also meet the holistic needs of patients, including their physical health care.

Secondary Care Challenges

- **Emergency Department Presentations**: Over the past few years, there has been a significant increase in the number of people presenting to Hamad General Hospital Emergency Department in mental health crises.

  This indicates a growing demand for mental health services post COVID and highlighting the consequential impact on psychiatric care in emergency department presentations. Further analysis is necessary to gain a deeper understanding of the needs of the patient attending ED, particularly those patients who are already known to mental health services. Chart 10 below provides a visual representation of presentations to ED.

Chart 12: Mental health presentations in Hamad General Hospital Emergency Department



	Presentations	Male	Female	Qatari	Non-Qatari
2020	9,469	8,103	3,166	3,698	7,411
2021	11,586	8,556	5,377	4,916	8,712
2022	11,933	11,401	4,123	6,336	9,261
2023	14,871	14,553	5,287	8,150	10,619
2024 (Jan-Apr)	5,301	3,732	2,527	2,173	4,095

- **Psychiatric Decisions Units:** The absence of Psychiatric Decisions Units (PDU) at General Hospitals leads to several challenges, including prolonged wait times for psychiatric patients in the emergency department, delayed access to psychiatric care, disruption of emergency department operations, inefficient resource allocation, patient agitation, extended hospital stays, compromised care quality for acutely ill patients, and potential staff burnout.

Specialist Mental Health Service Challenges

- **Mental Health Bed Shortage**: The National Mental Health Strategic Plan (2013-2018) initially identified a requirement of 12.5 beds per 100,000 population.
  With a future population projection of 2.5 million by 2030, Qatar should have a minimum of 313 beds. Currently, there are 129 beds in the government sector, situated in unsuitable facilities.
  Recent international research suggests an optimal bed ratio of 60 beds per 100,000 population, with the lowest acceptable ratio being 30 beds per 100,000[1]. Considering Qatar's unique demographics, the recommended number of psychiatric beds is 20 per 100,000, indicating a severe shortage.

- **Consolidating Specialist Mental Health Services:** The demand for inpatient mental health care continues to grow, necessitating specialized beds across the system, including acute, sub-acute, forensic, CAMHS, long-term residential care for adults with severe enduring mental illnesses, perinatal mother and baby and intellectual disabilities. Currently, some of these beds are distributed across HMC Mental Health Service-managed beds in Mesaieed Hospital, Salwa Road, Al Wakra, Bayt Al Dhiyafah, and Rumailah Hospitals. However, this provision is not only insufficient but also geographically scattered, making it challenging to resource services adequately given the rising demand.

- **Women's Mental Health Services:** There has been a considerable amount of work to develop specialized women's mental health services in the past 3 years. Including expanding access to outpatient clinics, establishing a female inpatient unit, the Women Wellness unit in Bayit Al Dhiyafa and introducing a rapid access to services through the virtual women's mental health service. However, due to demand there is a need to develop a single point of access to services, a dedicated perinatal community mental health team, outpatient clinics in primary care, specialized consultation and liaison services and as mentioned above a dedicated mother and baby unit.

- **Purpose-Built Sub-Specialties:** There are no purpose-built facilities to cater to the inpatient needs of children and adolescents, forensic patients, adults with intellectual disabilities presenting with acute mental illness or challenging behaviors, older adults requiring specialized inpatient mental health care, and perinatal mother and baby units.

[1] Molecular Psychiatry: Minimum and optimal numbers of psychiatric beds: expert consensus using a Delphi process; Adrian P. Mundt et al; January 2022.

- **Waiting Times for Out-patient Clinics**: Data from HMC's specialist mental health outpatient consultation waiting times for adults in 2023 shows that 43% of non-Qataris and 24% of Qatari patients waited over 60 days for their first appointment. These extended waiting times create difficulties for patients and strain other parts of the system due to repeated patient representations.

- **Capacity Constraints in Out-patient Clinics:** There is growing demand for specialist outpatient clinics for adults, which has led to increased waiting times for new consultations. Data from HMC Mental Health Services between 2020 and 2023 shows that 11% of outpatient caseloads were admitted to inpatient services, a significant rise from 0.7% four to five years ago. To optimize care pathways, there is a need to understand the characteristics and needs in the remaining 89% of the outpatient caseload. Understanding the proportion of patients who have remained stable could help identify those who might benefit from transitioning their ongoing treatment from secondary to primary care.

System-Wide Challenges and Opportunities

- **Integrated Care:** Based on the outlined demand, there is a need to further strengthen integrated care across primary and secondary care services, ensuring that patients receive the right treatment at the right time and in the appropriate part of the system.

  According to the stepped care model, the least intrusive and most effective intervention should be delivered first, in the least restrictive and stigmatizing environment. There is a significant opportunity to enhance integrated models of practice in patient management, which can improve patient waiting times for outpatient appointments, enhance outcomes, and streamline care delivery.

- **Digitizing Access and Mental Health Treatment**: There is an opportunity to further digitize access and mental health treatment to improve access, reduce waiting times, and decrease stigma. By leveraging telehealth platforms, online psychological support, and digital self-help tools, there is an opportunity to provide timely and convenient mental health services to a broader population.

  This approach will minimize geographical barriers, reduce the burden on physical facilities, and allow patients to seek help in a more private and less stigmatizing environment. Additionally, digital solutions can enhance integration across primary and secondary care, ensuring that patients receive the appropriate level of care quickly and efficiently.

- **Psychology Services**: Waiting times to access psychology services across primary and secondary care are significant, with insufficient activity to demonstrate effective patient transitions between these levels of care. There is a pressing need to expand psychology services in both primary and secondary care.
  Additionally, there is an opportunity to participate in a national project for the development and implementation of evidence-based psychological therapies according to the stepped care model.

- **Recovery and Measuring Outcomes:** There is a critical need for the system to adopt a more recovery-oriented approach, focusing not only on the clinical treatment of mental health conditions but also on the overall well-being and reintegration of patients into their communities. To effectively measure success and outcomes, it is essential to utilize a standardized outcome monitoring tool. This tool will enable consistent tracking of patient progress, identify areas for improvement, and ensure that treatment strategies are aligned with the goal of long-term recovery.
- **Workforce:** Recruiting bilingual (Arabic and English) mental health professionals, including psychologists, presents significant challenges. The primary obstacle is the limited availability of qualified candidates who possess proficiency in both languages and the requisite professional credentials. Additionally, competition for these professionals is intense, as bilingual skills are highly sought after globally, particularly in regions with growing expatriate populations. Retention can be challenging, as many professionals often seek opportunities in countries that offer higher salaries and better benefits. We will work in collaboration with our key stakeholders to develop a robust and comprehensive workforce plan for mental health, aligning this to national workforce planning projects.

- **Mental Health Research:** There is a system-wide need for a coherent and integrated mental health research program. Such a program would enable collaboration, optimize resources, and ensure the application of evidence-based practices, thereby improving treatment outcomes and addressing the complex needs of diverse populations receiving mental health care.

Substance Use Disorder System: There is a need to further expand the capacity and quality of the SUD treatment system in line with WHO/UNODC standards, with the aim of increasing the number of individuals receiving treatment and improving their outcomes. This will be achieved through the proposed development of a National Plan for Substance Use Disorder Treatment and National Plan for Substance Use Disorder Prevention. This need arises from the growing demand for services and the increasing complexity of drug use across populations, highlighting the necessity for culturally appropriate and evidenced based treatment and prevention.

Our Ideal System and Future State

The stepped care model which has been adopted for Qatar to deliver mental health services is an evidence-based, staged system, comprising a hierarchy of interventions from the least to the most intensive, which can be matched to the individual's needs. It promotes person-centered care which targets the needs of the individual.

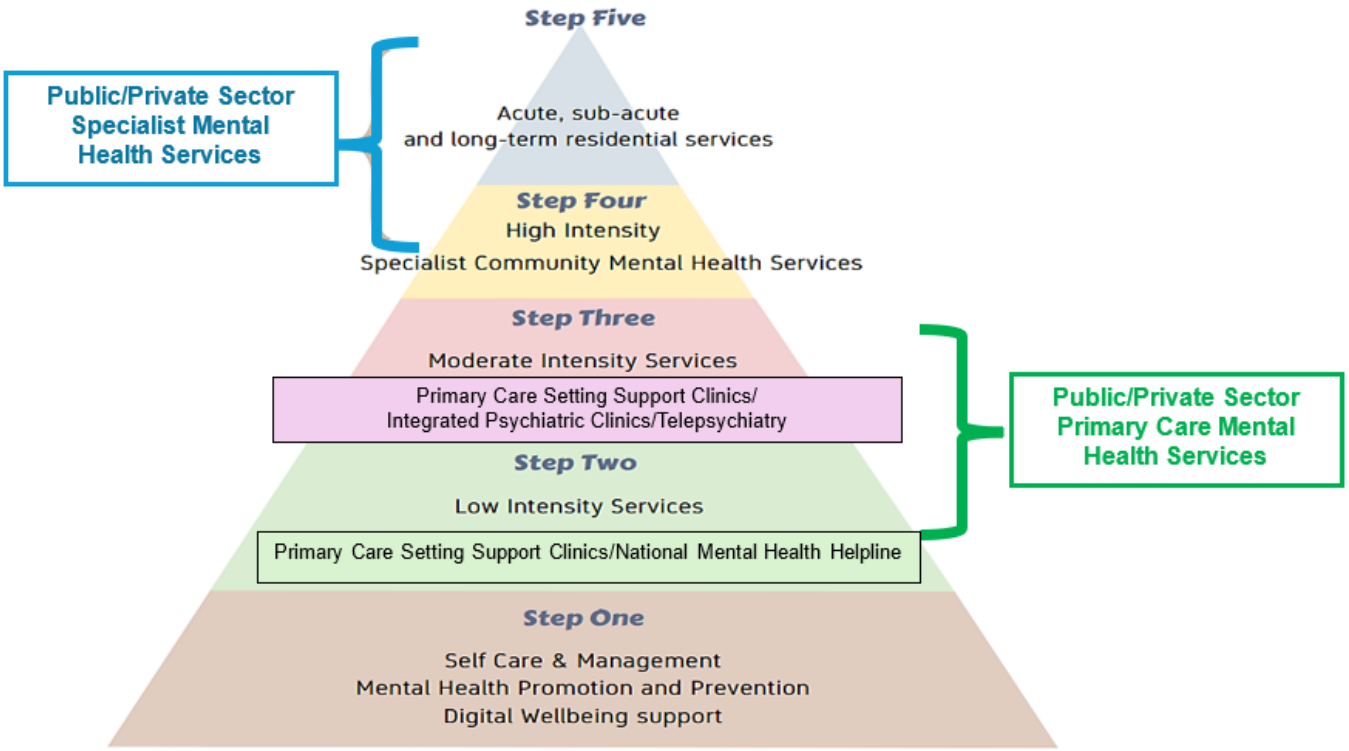
The model of stepped care should also be applied in psychological support services which should also follow a stepped care approach ensuring individuals receive the services that best meet their needs. Plans to revise the current psychological support services on this basis should be part of the ideal system moving forward.

Rather than offering a one size fits all approach to care, individuals will be more likely to receive a service that optimally matches their needs, does not under or over service them, and makes the best use of workforce and technology. A stepped care approach provides the right service at the right time, with lower intensity steps available to support individuals before illness manifests and support them to self-manage those conditions through the use of technology where it is clinically appropriate to do so.

Due to the continued pressure on the system, which is highlighted above, the stepped care system needs to be further augmented in order that it does not deliver 'top heavy' services and ensure that it remains in line with the overall vision for stepped care. Also, in line with the overall priority to continue the integration of services within primary care, the system should continue to work in partnership and remain committed to developing the range of services offered to reduce stigma and improve accessibility.

Outlined below is the Mental Health Stepped Model of Care, which describes Step 1 to Step 5 services. The details on the provision and specific projects that will be offered to achieve this are detailed in Section 3.

Design Note – Figure to be designed **Figure 4: Ideal State - Mental Health Stepped Model of Care**



Step 1: Self-care and management - promotes positive emotional health and wellbeing for all people in Qatar. Services are designed to prevent the onset of illness, or prevent further escalation, and are focused on supporting symptoms through self-management.

Step 1 includes mental health prevention, promotion, wellness services, wellbeing advice and information, including digital information and education.

Step 2: Low intensity services – these services cater to individuals with mild to moderate mental health conditions and are provided by primary health care family physicians, primary care and HMC psychologists (through Support Clinics), or via the National Mental Health Helpline. Designed for quick and easy access, these services do not require a formal referral. They are available through various modalities and typically involve a few short sessions.

Step 3: Moderate intensity services - offer structured, frequent, and intensive interventions tailored for individuals with mild to moderate mental health conditions.

These services are typically delivered by primary health care family physicians, primary care and HMC psychologists (operating through Support Clinics) or accessed via the National Mental Health Helpline. In cases of moderately severe conditions, patients are managed by PHCC's integrated psychiatric clinics, which are operated by PHCC and HMC Psychiatrists in collaboration with primary care clinicians.

Step 4: High intensity services – services cater to individuals with severe mental illness, which may be persistent or episodic, but not accompanied by a high level of risk or complexity. These services are predominantly provided by HMC specialist mental health services in community outpatient settings, backed by multidisciplinary staff. Additionally, these services encompass outreach and home treatment options.

Step 5: Acute, sub-acute & long-term residential services – encompass inpatient and residential services designed for individuals with severe mental illness and/or complex needs. These services are often utilized for stabilizing acute symptoms of mental illness and for rehabilitation prior to discharging patients back into the community. They involve intensive, team-based specialist assessment and intervention, incorporating expertise from a diverse array of mental health professionals which may include private providers. The specialist provision required for Qatar is set out in the National Mental Health Integrated Facilities and Bed Plan 2023/203

SECTION 3: ACTION PLAN MENTAL HEALTH PROJECTS

IMPROVING ACCESS Design Note: Create icon for Improving Access

We will focus on making mental health services more accessible and less stigmatizing.

Our key projects include creating a digital gateway for mental health information, providing digital access to the helpline, and expanding the National Mental Health Awareness Program. We will enhance integrated mental health and substance use disorder services in primary care by increasing the number of integrated psychiatric clinics and implementing the SBIRT program. We will also develop mental health-friendly hospital emergency departments and Psychiatric Decision Units in general hospitals. Additionally, we will collaborate across sectors, including educational institutions, workplaces, and community organizations, to promote mental health awareness, reduce stigma, and provide integrated support systems.

EXCELLENCE IN SERVICE DELIVERY

Design Note: Create icon for Excellence in Service Delivery

We aim to enhance mental health services by developing primary and specialized care, integrating service delivery, and adopting patient-centered approaches. The plan includes consolidating specialist mental health services and integrating care models across various levels to ensure seamless patient journeys. Emphasis is placed on patient-centered care, tailoring services to individual needs and promoting empowerment. The plan also focuses on strengthening primary and community care, developing sub-specialties like child and adolescent mental health, and expanding access to evidence-based psychological therapies through a national program, available both in person and via digital means.

MENTAL HEALTH SYSTEM EFFICIENCY & RESILIENCE

Design Note: Create icon for Mental Health System Efficiency & Resilience

We will focus on creating a dynamic and responsive mental health system through the "Mental Health System Efficiency and Resilience" priority. Our key initiatives include workforce planning and development to enhance staff recruitment, retention, and satisfaction. We will implement data-driven decision-making by establishing a unified mental health data set and monitoring performance and outcomes. We will ensure sustainable healthcare financing by shifting to program-based, ring-fenced budgets, promote innovation and research, including expanding our mental health research and genome sequencing capabilities. Lastly, we will achieve efficiency by providing the right care in the right location, focusing on locating the majority of care in the community.

In order to achieve the desired outcomes over the next six years, we will collaborate with a range of internal and external partners.

Outlined below is our Action Plan highlighting our key projects including lead organizations, and timelines to drive the transformation program across the system. Detailed project plans will be developed for each deliverable including comprehensive key performance indicators and outcome measures to ensure our plans are on track and we are making a positive impact on the mental health system in Qatar.

Additionally, to monitor the progress of the plan's implementation, we will establish appropriate monitoring and reporting mechanisms through a dedicated governance structure.

Action Plan Mental Health Projects & Milestones

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
1. Improve Public Awareness and Health Literacy					
1.1	National campaign to improve mental health literacy and reduce stigma and discrimination, involving people with lived experiences.	Collaborating with key stakeholders and people with lived experience of mental health conditions. Develop and deliver a national campaign to improve health literacy.	MoPH	National Anti-Stigma Campaign developed and rolled out Q1 2026-Q4 2026	5.7.1 Whole population Mental Health Access and Integration
1.2	Expansion of the National Mental Health & Awareness Program Ma'ak to enhance and target key sections of the population. Including government ministries and other government entities.	Identify key population groups and their needs. Expanding the Ma'ak Program resources and training materials.	MoPH	Development and roll out of 3 targeted mental health & awareness programs Q1 2026-Q4 2027	5.7.1 Whole population Mental Health Access and Integration
1.3	Develop National SUD prevention and education campaigns.	In collaboration with key stakeholders and evidence-based practice. Develop and deliver a range of substance use disorder prevention and education materials and campaigns.	MoPH	National Substance Use Disorder Campaign developed and rolled out Q1 2026-Q4 2027	5.7.2 Substance Use
1.4	National wide campaigns for the prevention of suicide and self-harm	Develop and deliver a range of national wide campaigns for the prevention of suicide and self-harm based on international best practice.	MOPH	National Prevention of Suicide and Self-Harm Campaign developed and rolled out Q1 2027-Q2 2028	5.7.2 Substance Use

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
2. Advance Universal Healthcare Access and Enhance Person-Centered Care					
2.1	Expansion of substance use disorder screening in primary care	Expand substance use disorder screening and brief interventions (SBIRT) in PHCC Health Centers following training of family physicians, nurses, and psychologists.	PHCC	80% of adults visiting Health Centers to be screened for substance use disorders Q1 2025-Q4 2027	5.7.2 Substance Use
2.2	Development of a Mental health friendly emergency department program.	This will equip Emergency Department staff to identify patients experiencing mental health crises, help them manage distress, and facilitate referrals to appropriate services.	HMC	Mental Health Emergency Department Program developed and rolled out in all HMC EDs by Q1 2026-Q4 2028	5.7.1 Whole population Mental Health Access and Integration
2.3	Development of screening and brief interventions and referral to treatment for substance use disorders in Emergency Departments.	Implement the SBIRT model currently delivered in PHCC to identify and treat patients using substances who attend Emergency Department.	HMC	SBIRT Model implemented in all HMC EDs Q1 2027-Q4 2027	5.7.2 Substance Use
2.4	Establish Child and Adolescent Mental Health Inpatient service.	Development of a dedicated inpatient unit for young people supported through a managed clinical network of stakeholders. This will include support for eating disorders.	MoPH/ Sidra	CAMHS Inpatient Unit operational Q1 2025-Q1 2026	5.7.1 Whole population Mental Health Access and Integration
2.5	Expansion of Psychology Support Clinics in primary health care.	Increase the number of support clinics in PHCC for patients with low to moderate need. In addition to increase of psychology services across the private sector.	PHCC	Increase the number of support clinics to 9 Health Centers Q1 2025-Q4 2026	5.7.1 Whole population Mental Health Access and Integration

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
2.6	Consolidate and streamline Specialist Mental Health Inpatient Services in line with the National Mental Health Integrated Facilities Bed Plan	Commission, build, and operationalize efficient inpatient services moving away from geographically dispersed provision. In addition to the development of integrated pathways.	HMC	80% of bed requirements identified in national bed plan operationalized Q1 2026-Q4 2030	5.7.1 Whole population Mental Health Access and Integration
2.7	Roll out a national stepped care program to improve the accessibility and delivery of evidence-based psychological therapies.	Develop a National Psychology Program which includes the development and implementation of the stepped care model and improved training/education and career progression for psychology staff	HMC/ PHCC	National Psychology Program developed, referral pathways established and implemented Q2 2025-Q4 2028	5.7.1 Whole population Mental Health Access and Integration
2.8	Establish Mental Health Crisis Intervention Services.	Establish a rapid response system for Qatari nationals experiencing mental health crises, with the goal of diverting them from the ED when appropriate.	HMC	Mental Health Crisis Intervention Services for Qatari nationals Q4 2025-Q4 2026	5.7.1 Whole population Mental Health Access and Integration
2.9	Establish an intellectual disability mental health inpatient service.	Commission, build, and operationalize a dedicated inpatient unit specifically designed for patients with intellectual disabilities requiring short-term admission for mental health stabilization.	MOPH/ HMC	Intellectual Disability Inpatient Service for patients at risk to themselves or others operational Q4 2026-Q4 2028	5.7.1 Whole population Mental Health Access and Integration
2.10	Expand Child & Adolescent Mental Health services in primary healthcare.	Ensure the development of CAMHS primary care settings for children aged 0 to 18 providing access to and treatment of common mental health conditions.	PHCC	CAMHS developed, referral pathways between primary and secondary care established and operational across primary care settings Q1 2025-Q4 2027	5.7.1 Whole population Mental Health Access and Integration
2.11	Expand long-term residential rehabilitation services for people with severe enduring mental illness.	Residential rehabilitation bed capacity increased for patients with severe enduring mental illness in a residential facility.	MOPH/ HMC	Residential bed capacity for severe enduring mental illness increased Q2 2025-Q2 2026	5.7.1 Whole population Mental Health Access and Integration

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
2.12	Development of peer support services within community settings, for people with mental health conditions and substance misuse.	Develop peer support networks with patients with lived experience to support patients to remain engaged in services and enhance patient experience.	HMC, Naufar	Peer Support Program developed and implemented including governance arrangements Q4 2025-Q42 2027	5.7.1 Whole population Mental Health Access and Integration 5.7.2 Substance Use
2.13	Develop a transition plan to achieve further integration of mental health services into primary health care ensuring the right care is delivered in the right location.	Transition specialist outpatient clinics from HMC to PHCC health centers as part of the Salwa Road decanting initiative and expanding the number of integrated psychiatric clinics within health centers.	PHCC/ HMC	Transition Plan for Integration of Mental Health Services implemented Q1 2025-Q4 2025	5.7.1 Whole population Mental Health Access and Integration
3. Strengthen Information Management and Research Capabilities					
3.1	Develop a comprehensive research program for mental health services	In collaboration with key stakeholders, develop a comprehensive research plan for mental health services. Utilizing the research findings to inform the development of future services for mental health.	MoPH	National Mental Health Research Program developed Q1 2026 -Q4 2026	5.7.1 Whole population Mental Health Access and Integration
3.2	Enhance Information systems, data governance, analytics and business planning (unified datasets, reporting and needs assessments to inform future planning.	Working across the government and private sector services establish a unified system to collate, analyze, monitor and report mental health and substance use information to support future planning of services.	MoPH	Dedicated Mental Health Information System developed Q4 2025-Q2 2026	5.7.1 Whole population Mental Health Access and Integration
3.3	Develop and implement quality standards for mental health and substance use	In collaboration with key stakeholders develop and implement quality standards across the services to ensure clinical quality standards are being adhered to across all mental health and substance use services in the country.	MOPH	Quality Standards for Mental Health and Substance Use Disorders developed and implemented Q1 2026-Q4 2026	5.7.2 Substance Use

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
4. Accelerate Digital Health Integration and AI utilization					
4.1	Development and implementation of universal and targeted digital self-help interventions.	In partnership with key organizations, develop and implement self-help materials Which can be utilized across digital platforms.	MoPH	Universal Digital Self-Help Interventions developed and implemented Q4 2027-Q4 2030	5.7.1 Whole population Mental Health Access and Integration 5.7.2 Substance Use
4.2	Digitization and expansion of the National Mental Health Helpline.	In order to improve access to the national mental health helpline. Ensure access is available through digital platforms.	HMC/ MoPH	Increase in utilization rate of the mental health helpline by 40% from baseline within 24 months following the launch of the digitized platform and targeted campaign. Q4 2026-Q4 2028	5.7.1 Whole population Mental Health Access and Integration
4.3	Digitization and expansion of targeted campaigns for young people to improve access ensuring access is available through digital platforms.	Develop a timetable of targeted campaigns for young people each year to reduce stigma and promote early intervention and prevention.	MoPH	Developed and rolled out at least 3 targeted campaigns Q1 2028-Q4 2029	5.7.1 Whole population Mental Health Access and Integration
4.4	Establish partnerships with digital and AI experts to revolutionize the mental health care system.	Working with educational institutions and experts in artificial intelligence. Develop partnerships to deliver mental health services in new and innovative ways.	MOPH	Minimum of 1 partnership established with digital health and AI organization Q4 2026-Q1 2028	5.7.1 Whole population Mental Health Access and Integration
4.5	Enhance Data Management and Data Sharing and automate processes of administrative and clinical data.	Utilizing technology streamline the processes associated with the data management of mental health and substance use services.	MOPH	Automated Data Management System developed and implemented Q1 2026-Q4 2026	5.7.1 Whole population Mental Health Access and Integration

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
5. Advocate for Mental Health Policies and Engage Stakeholders					
5.1	Expansion of support for families and carers, including development of patient engagement and Advocacy Plan to support Patients, Families and Carers.	Develop a clear and comprehensive patient engagement and advocacy plan. To ensure patients with mental health conditions are supported during their treatment and after discharge into the Community. In addition to ensuring families and carers are provided with the support they need.	MOPH	Patient Engagement and Advocacy Plan developed Q1 2028 -Q2 2030	5.7.1 Whole population Mental Health Access and Integration
5.2	Develop a national plan for the treatment of substance use disorders.	Develop a comprehensive National Plan for the treatment of substance use disorders in collaboration with key stakeholders. The plan will outline the key services and interventions required to support people who have a range of substance use disorders in the country.	MOPH/ HMC/ Naufar	National Plan for the Treatment of Substance Use Disorders developed and published Q1 2026- Q4 2026	5.7.2 Substance Use
5.3	Family Support Program Development: Implementation of support for families and carers including development of patient engagement and advocacy plan to support patients, families and carers	Identify all relevant stakeholders, including patients, families/caregivers, mental health professionals, and community organizations. Conduct focus groups and/or surveys to gather insights on their experiences, needs, and expectations related to mental health services.	MOPH	Family Support Program developed and implemented Q1 2028-Q2 2030	5.7.1 Whole population Mental Health Access and Integration
5.4	Identify vulnerable at-risk families through collaboration with other entities and provide access to mental health services with a holistic view of all family members	Form strategic alliances with cross-organizational healthcare providers (public and private) and identified community organizations to create a network that identifies at-risk families.	MOPH	Strategic Alliances developed Q1 2026-Q4 2027	5.7.1 Whole population Mental Health Access and Integration

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
6. Enhance the Mental Health Infrastructure and Empower Health Professionals					
6.1	Develop advanced mental health knowledge & skills training programs for primary health care. Including mentorship programs	Develop a CPD accredited mental health knowledge and skills training program for new clinical staff joining PHCC	PHCC	CPD Accredited Mental Health Skills Training Program developed and rolled out Q1 2026- Q4 2026	5.7.1 Whole population Mental Health Access and Integration
6.2	Develop a comprehensive mental health training program for Urgent Care Center Physicians in primary care.	Develop a mental health training program for Urgent Care physicians and nurses in primary care, covering assessment, crisis management, screening, and referral pathways, with a focus on culturally sensitive, integrated patient care.	PHCC/ HMC	Mental Health Training Program for Urgent Care staff developed and rolled out Q1 2026-Q4 2026	5.7.1 Whole population Mental Health Access and Integration
6.3	Expansion of perinatal mental health services, including the establishment of a mother baby unit.	Expand perinatal mental health services with a dedicated mother-baby unit and enhanced community-based care, supporting mothers' mental health through accessible, integrated treatment options and specialized support for families.	HMC	Mental Health Mother and Baby Unit operational Q3 2026- Q4 2028	5.7.1 Whole population Mental Health Access and Integration
6.4	Establish a healthy aging acute mental health unit.	Establish an acute mental health unit for healthy aging, focused on short-term stabilization of functional mental illness, providing specialized, supportive care tailored to the needs of older adults.	HMC	Healthy Aging Acute Mental Health Unit operational Q2 2027-Q4 2029	5.7.1 Whole population Mental Health Access and Integration
6.5	Establish and operationalize a dedicated forensic mental health hospital and integrated forensic mental health outpatient services.	Establish and operationalize a specialized forensic mental health hospital alongside integrated forensic mental health outpatient services, providing comprehensive assessment, treatment, and rehabilitation tailored to individuals in the justice system.	HMC	Forensic Mental Health Hospital operational Q2 2025-Q4 2029	5.7.1 Whole population Mental Health Access and Integration

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
6.5	Establish and operationalize a dedicated forensic mental health hospital and integrated forensic mental health outpatient services.	Establish and operationalize a specialized forensic mental health hospital alongside integrated forensic mental health outpatient services, providing comprehensive assessment, treatment, and rehabilitation tailored to individuals in the justice system.	HMC	Forensic Mental Health Hospital operational Q2 2025-Q4 2029	5.7.1 Whole population Mental Health Access and Integration
6.6	Establish programs to enhance the mental health and wellbeing of the healthcare workforce.	Implement programs to promote and support the mental health and well-being of the healthcare workforce, fostering resilience, reducing burnout, and creating a supportive environment for staff through targeted interventions and accessible resources.	HMC/ PHCC	Employee Mental Health & Wellbeing Programs developed and rolled out Q4 2025-Q2 2026	5.7.1 Whole population Mental Health Access and Integration
6.7	Identify psychiatric institutions whose infrastructure can be transformed into centers of excellence for teaching, research, and the promotion of integrated care.	Identify psychiatric institutions with potential for transformation into centers of excellence, focusing on teaching, research, and integrated care to advance mental health services, academic growth, and collaborative care models.	HMC/ MOPH	At least 1 Mental Health Center for Excellence established Q1 2027-Q4 2029	5.7.1 Whole population Mental Health Access and Integration
6.8	Establish dedicated Psychiatric Decision Units (PDU's) within general hospital settings.	Develop and operationalize a PDU in each identified general hospital settings to support access to treatment.	HMC/ MoPH	A PDU developed and operational in each general hospital Q1 2026-Q4 2030	5.7.1 Whole population Mental Health Access and Integration
7. Expand Multi-Sectoral and International Collaborations					
7.1	Collaboration partnership with MOI to address the needs of the prison population with mental health conditions.	Develop a partnership with MOI to address the mental health needs of the prison population, as part of the scoping and implementation of the forensic hospital initiative.	HMC/ MOI	Partnership/MOU established between HMC and MOI Q2 2026-Q4 2029	5.7.1 Whole population Mental Health Access and Integration
7.2	Development of mental health curriculum within local universities to enhance capacity building of workforce	Develop mental health curriculum with local universities to build workforce capacity, equipping professionals with essential skills and knowledge.	MOPH/ HMC	National Mental Health Curriculum developed and implemented Q4 2026- Q3 2027	5.7.1 Whole population Mental Health Access and Integration

Project		Brief Description	Lead Entity	Strategic Key Performance Indicator	NHS Initiative
7.3	Map current Mental Health provision identifying opportunities for public private partnerships. Develop a comprehensive Public Private Partnership Plan in line with future service needs.	Develop a comprehensive Public-Private Partnership plan aligned with future mental health service needs, identifying key areas for PPPs and establishing governance structures to ensure effective collaboration and oversight.	MOPH	Mental Health Public-Private Partnership Plan developed Q2 2025- Q4 2025	5.7.1 Whole population Mental Health Access and Integration
7.4	Enhance local strategic networks	Developing managed Mental Health clinical networks across organizations and strengthening alignments.	MOPH	Mental Health Clinical Networks developed Q1 2026-Q4 2026	5.7.1 Whole population Mental Health Access and Integration
7.5	Enhance Regional and international Coordination	Enhance regional clinical and non-clinical professional networks. Utilizing best practice across the MENA region.	MOPH	Increase in the number of regional and international networks Q1 2026-Q4 2026	5.7.1 Whole population Mental Health Access and Integration

SECTION 4: HOW WILL WE TRANSLATE STRATEGY INTO ACTION?

Measuring and reporting on progress in implementing the Plan is essential to ensuring we are making a difference not only to the current service but also to inform future planning.

Our Measures of Success not only support the key strategic mental health priorities outlined in this Plan but will also make a vital contribution to achieving the key outcomes of the QNHS 2024-2030 and other initiatives and plans. They will also make a significant contribution to improving mental health and wellbeing outcomes across our health partner organizations objectives related to mental health and substance use disorders.

To ensure successful implementation, the Plan will be supported by a number of key enablers, including robust performance management mechanisms to support information and research initiatives; training and development programs and clear workforce plans to support current and future service developments.

Key Project Implementation Plans will be developed and will clearly articulate the key actions for mental health over the next six years. Each action plan will outline a summary of the priorities and initiatives which will be undertaken and the success measures associated with each initiative to demonstrate how priorities will be met.

Under each project a “responsible entity’ organization has been identified who will be responsible for engaging and coordinating the relevant stakeholders to ensure successful implementation of each project. To support this approach a number of action task groups will be established to manage, implement and monitor progress against each project. Each of the assigned action task group leads will be responsible for reporting progress to the relevant oversight structures within the QNHS 2024-30.

Implementing this Plan will require strong leadership, commitment, an integration approach and key resources to support delivery.

We need to address many challenges which still exist in terms of stigma but also ensuring the right workforce is in place to deliver our service plans. We will continue to collaborate more broadly and ensure we continue to develop effective programs and interventions for the successful transform of the mental health system across Qatar.

ANNEX 1: Acknowledgements

We would like to extend our thanks to all of those who supported the development of this plan. Specifically, the key authors and contributors of the plan are outlined below.

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ANNEX 2: Abbreviations

AI: Artificial Intelligence

ADHD: Attention Deficit Hyperactivity Disorder

CAMHS: Child and Adolescent Mental Health Services

CL: Consultation Liaison

COVID: Corona Virus Disease

ED: Emergency Department

HMC: Hamad Medical Corporation

MOI: Ministry of Interior

QNHS 2024-2030: National Health Strategy 2024-2030

PDU: Psychiatric Decisions Unit

PHCC: Primary Health Care Corporation

QNMHS: Qatar National Mental Health Strategy

QNDS: Qatar National Development Strategy

QNHS: Qatar National Health Strategy

QNV: Qatar National Vision

SBIRT: Screening, Brief Interventions, and Referral to Treatment

SUD: Substance Use Disorder

UNODC: United Nations Office on Drugs and Crime

WHO: World Health Organization

ANNEX 3: References

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